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Name: نوال عطوه دسوقي النقيب National ID: 25702220100404	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
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المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين


 جمهورية مصر العربية
 وزارة الداخلية
 إدارة المرور
 كارت رخصة قيادة
 السيد / محمد علي النجدي
 ٢٥٧٠٢٢٠١٠٠٤٠٤
 KE7137497

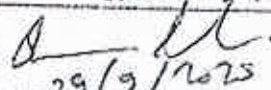
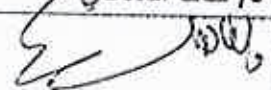
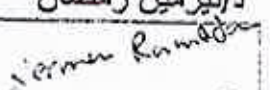

 ٢٠٢٣/١٠
 ٥٧٠٢٢٠١٠٠٤٠٤
 رخصة مرور
 السيد / محمد علي النجدي
 المنطقة سكنية حي النجدي
 ٢٠٣٠/١٠



Patient Name	نوال عطوة دسوقي	Age	68
Sex	F	Special habits	
Family History	+ve	Comorbidity	Liver cirrhosis +CLD
Surgical History	Total hysterectomy	Telephone	0122919644
Menopausal Status	post	Marital Status	widow
ID	2570220100454	Medical ID	1856/2025
Date	29/9/2025	Reason of MDT	

Medical Report	
Pathology: PT1c, PN1a (2/16) post Rt BCS+SLNB	
ER: +ve	
PR: +ve	
HER2: -ve	
Ki67: 3%	
T N M	
S.M:	
Metsworkup:	
Recommendations : upfront Rt BCS + ALND	

MDT DECISION	In view of having N1 (2/16) with extra capsular extension. FOR: 1. Adjuvant short course Cth (TC x4) with liver support 2. PORT to Right breast chest wall and PCLNs 3. ET for at least 5 years (aramidex) + zometa every 6 months
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Name	د/شيماء لاشين	د/ خالد السعدي	د/نيرمين رمضان
Signature			

Patient Name : الأستاذ / نوال عطوده دسوقي النقيب
Patient No. : 25702220100404
Path No. : 2737/25
Sex : Female
Age : 68 Y
Referring Doc :

Request No. : 13006660
Request Date : 03/09/2025
Verification Date : 13/09/2025
Financial Category : جوستاف روسي - نفقة الدولة
Department : Out Patient
Registered by : Mrs. Mayada Hegazy

Pathology Report

Clinical Data: Right breast cancer, Up-Front.

Nature Specimen: Right breast conservative surgery, Extra-lateral margin, Extra-inferior margin, Extra-deep margin, Sentinel lymph nodes biopsy & Completion axillary clearance.

Gross Examination:

Six specimens were received as follow:

- Right breast conservative surgery specimen measured 09x07x3.5 cm, with covering skin ellipse measured 5.5x2 cm.
- * Serialling showed a mass 1.8x1.2x1 cm, located 0.1 cm from lateral margin, 0.5 cm from medial margin, 1.5 cm from superior margin, 3 cm from inferior margin, 2 cm from skin & 5 cm from the deep margin.
- Extra-lateral margin: Fibrofatty tissue piece measured 3.5x3x0.5 cm, looked unremarkable grossly.
- Extra-inferior margin: Fibrofatty tissue piece measured 4x3x1 cm, looked unremarkable grossly.
- Extra-deep margin: Fibrofatty tissue piece measured 3x3x1 cm, looked unremarkable grossly.
- SLNs: Multiple fibrofatty tissue pieces collectively measured 3x3 cm, entangling 4 lymph nodes, the largest was 1.5x1x0.5 cm.
- ALNs: Axillary fat 5x4.5x3 cm, entangling 12 lymph nodes, the largest was 2x1x0.5 cm.
- Total nodal number: 16 lymph nodes.

15 Blocks:

- 1:2- Mass., 3- Medial margin., 4- Lateral margin., 5- Superior margin., 6- Inferior margin., 7- Deep margin., 8- Skin., 9- Extra-lateral margin., 10- Extra-inferior margin., 11- Extra-deep margin., 12- SLNs., 13:15- ALNs.
- Gross was done by; Dr. Mennan Hossam.

Verified by :

Dr. Duaa Mohamed

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غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name : الأستاذة / نوال عطوه دسوقي النقيب
Patient No. : 25702220100404
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Age : 68 Y
Referreing Doc :

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Pathology Report

Frozen (Intra-operative) Results:

- Suspicious (Lateral, inferior & deep) margins for DCIS.
- Free other surgical margins.
- SLNSs: Positive metastatic deposits in two out of four sentinel lymph nodes (2/4) with extra-nodal extension.

Microscopic Examination:

- * Histological Type: Invasive duct carcinoma.
- * Grade 2 (differentiation 2, nuclear pleomorphism 2, mitosis 2).
- * 5% Intermediate grade ductal carcinoma in situ (Solid & cribriform patterns with comedo type necrosis).
- * Angioinvasion: Present.
- * Perineural invasion: Absent.
- * Microcalcifications: Present.
- * Skin: Free.
- * Surgical margins: Free extra-lateral margin & free extra-inferior margin, free extra-deep margin & free all other surgical margins.
- * Sentinel lymph nodes: Positive metastatic deposits in two out of four sentinel lymph nodes (2/4) with extra-nodal extension.
- * Completion axillary clearance: Twelve free lymph nodes (0/12).
- * Total nodal status: Positive metastatic deposits in two out of sixteen lymph nodes (2/16) with extra-nodal extension.

Diagnosis:

Right breast cancer (Up-front), Right breast conservative surgery, Extra-lateral margin, Extra-inferior margin, Extra-deep margin, Sentinel lymph nodes biopsy & Completion axillary clearance:

- Invasive duct carcinoma, Grade 2, with 5% Intermediate grade ductal carcinoma insitu

Verified by :

Dr.Duaa Mohamed

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Financial Category : جوستاف روسي - نفقة الدولة
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

- (Solid & cribriform patterns with comedo type necrosis).
- Resection Margins: Free extra-margins & all-other surgical margins.
- Skin: Free.
- Axillary lymph nodes: Positive metastatic deposits in two out of sixteen lymph nodes (2/16) with extra-nodal extension.
- Stage: pT1c, pN1a.

Signature
Prof. Dr. Gamal Abd-El-Aty.
Dr. Mennan Hossam.

Verified by :
Dr.Duaa Mohamed

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Clinical summary 68-year-old, right breast palpable lump for 1 month

Technique

Medio-lateral oblique and CC views. High resolution ultra sound

Detailed Findings

Right breast

- Scattered fibro glandular parenchyma, (ACR B)
- UOQ well defined dense mass with irregular margin
- No suspicious micro calcifications.
- Ultrasound demonstrated a corresponding hypoechoic solid mass with irregular and angular margin as well as internal vascular signal depicted by color Doppler study, it is seen at between 10-12 o'clock zone c about 11cm from the nipple and 5cm from the skin, measuring 18x15mm
- Normal skin/subcutaneous tissue and nipple/areolar complex.
- Prominent retroareolar duct measuring 2mm in caliber with clear pattern
- Enlarged one pathological axillary node measures with non-specific axillary LNs seen at level I

Left breast

- Scattered fibro glandular densities (ACR density B).
- Retroareolar well defined dense mass with regular margin
- No asymmetric densities, distortion or suspicious micro calcifications.
- Ultrasound at retroareolar shows well defined hypoechoic solid lesion with regular border and no internal vasculature by color doppler
- no cystic lesion
- Normal skin/subcutaneous tissue and nipple/areolar complex
- Non-specific axillary LNs.

Impression:

Right breast (BIRADS 5): UOQ palpable malignant looking solid mass

left breast (BIRADS 3): benign looking solid lesion

Recommendations: Biopsy was done on the same sitting from right breast

Dr. Marwa M.Hassan

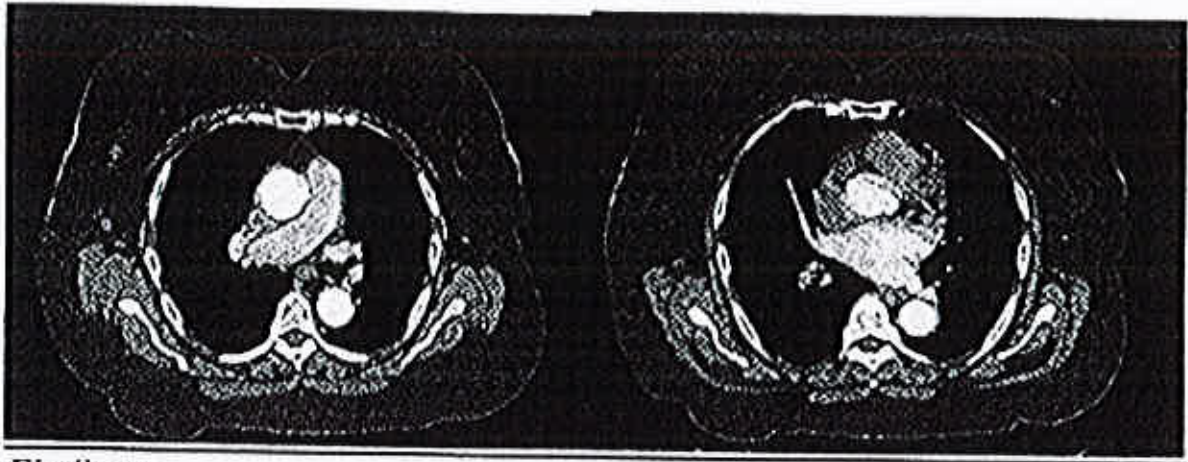
Name: Nawal Atwa

Date: 20-7-2025

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST: -
PRIMARY REPORT**

Clinical History:

- Known case of breast cancer, no available previous films for comparison.



Findings:

- Right breast outer quadrant speculated soft tissue lesion is noted measuring about 12.5x9.5 mm with overlying skin thickening reaching 7 mm.
- Few suspicious looking right axillary lymph nodes are noted, the largest seen measuring 12.5x9.5 mm.
- Left breast outer quadrant tiny well defined lesion measuring about 8x6 mm as well as skin thickening reaching 6 mm for sonomammography correlation.
- Few left axillary lymph nodes are noted, one of them seen rounded in shape with lost fatty hilum measuring about 7x6 mm for sonomammography correlation.
- Both lungs show normal appearance of its parenchyma with no sizable pulmonary nodules, masses or areas of consolidation.

- Few bilateral scattered atelectatic bands are noted.
- No sizable hilar or mediastinal lymph nodes.
- Normal CT appearance of the mediastinal vessels.
- Patent tracheo-bronchial tree.
- Average size of the heart.
- No evidence of pleural or pericardial effusion.

OPINION:

- *Right breast outer quadrant speculated soft tissue lesion with overlying skin thickening and few suspicious looking right axillary lymph nodes for further assessment.*
- *Left breast outer quadrant tiny well defined lesion as well as skin thickening and few left axillary lymph nodes for sonomammography correlation.*